



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : انوار محمد متولي ابوكريم
المحافظة : الجيزة
الحى / المدينة : البدرشين
رقم البطاقة : 27509262100601 رقم التليفون : 01028462746
العنوان : -----
العمر (تاريخ الميلاد) : 26-09-1975 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: انوار محمد متولي ابو كرم National ID: 27509262100601	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

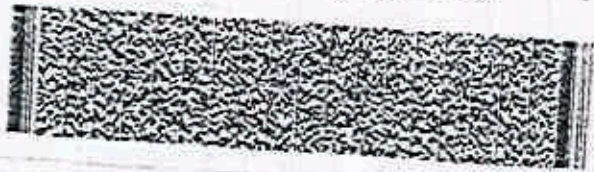


٢٧٥٠٩٢٦٢١٠٠٦٠١

ر.م.م.م.



مستند
شكر محمد موسى محمد ابو كريمة
المنطقة سرية حتى ٢٠٣٠/١٠٣/٢٣



أمانة مركز البحوث والبحوث
م.م.م.م.

النواير

محمد متونى ابو كريمة

قرية سقارة

مركز البحوث والبحوث - الجيزة



٢٧٥٠٩٢٦٢١٠٠٦٠١

JY3596721

Name : Anwar Mohamed Metwally Abu Karim
Branch : Maadi
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 22-07-2025 01:44
Medical No. : 157805
Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 50-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Diffuse enhanced tracer uptake is noted at the sternal body.
- No other abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- No frank evidence of metastatic bone disease.
- Equivocal tracer uptake pattern at the sternum, for clinical/radiological assessment and follow up.

Much obliged
Dr. Mohammed AtaAllah, MSc
Nuclear Medicine
Dr. Alaa Nagi, MD
Consultant of nuclear medicine

Patient Name: Anwaar Mohamed Metwally Abu Karim	Date : 31/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 4946/2021	Pat. ID: 16342

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 50 years old female with right breast pathologically proven breast cancer, on neoadjuvant hormonal therapy. No available radiological data for comparison

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ, middle third, irregular speculated high density mass lesion with surrounding distortion.
- Central inner distortion and partially visualized irregular mass (out of field).
- No suspicious microcalcifications.
- Ultrasound demonstrated 10 o'clock middle third, irregular spiculated heterogenous hypoechoic solid anti-parallel mass lesion with avid surrounding vascularity on color Doppler study measuring 8 x 6 mm.
- Another similar lesion is seen at 3 o'clock, posterior third near the mid line measuring 11 x 8 mm.
- Normal skin/nipple/areola complex.
- One pathological axillary LN, level 1 is noted showing focal cortical thickness reaching 4.5 mm and hilar indentation, it measured 11 x 6 mm.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast parenchyma with mottled appearance yet no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Patient Name: Anwaar Mohamed Metwally Abu Karim	Date : 31/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 4946/2021	Pat. ID: 16342

Conclusion:-

Right breast (BIRADS 6): Multi-centric pathological process with ipsilateral one pathological level 1 lymph node. (No available radiological data for comparison)

Left breast (BIRADS 1): Normal study.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

Suspected Cancer Risk

BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Not Approved

Patient Name: Anwaar Mohamed Metwally Abu Karim	Date : 28/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 4946/2021	Pat. ID: 16342

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast mild diffuse skin thickening.
- Bilateral axillary lymph nodes are noted largest measuring 14 mm at the left axillary region.
- Few bilateral pulmonary nodules largest measuring 5 mm at the left lower lobe
- Calcified mediastinal lymph nodes are seen.
- Mild bronchiectatic changes are noted at the apical segment of the left lower lobe.
- Few tiny bilateral peripheral emphysematous bullae.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions on non-contrast basis or contour irregularities.
- Calcular gall bladder.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Multiple subcentimeteric mesenteric and para-aortic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra-renal glands.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

Patient Name: Anwaar Mohamed Metwally Abu Karim	Date : 28/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 4946/2021	Pat. ID: 16342

- No abdominal or pelvic masses or collections.
- **Multiple dorsal and lumbar small lytic areas as well as D4 and S1 small sclerotic areas are noted for bone scan.**

Dr. Sherif Wagdi



Patient Name: Anwaar Mohamed Metwally Abu Karim	Date : 19/03/2025
Exam : C.T CHEST	Age : 49 Year(s)
Medical ID: 4946/2021	Pat. ID: 16342

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- Compared to previous study dated 14/10/2024.

1. Chest:

- **stable course** regarding the right breast diffuse skin thickening.
- **Still noted:**
 - Prominent subcentemetric right axillary level I and II lymph nodes.
 - Few bilateral pulmonary nodules ranging from 2-4 mm (insignificant).
- Calcified mediastinal lymph nodes are seen.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions on non-contrast basis or contour irregularities.
- Calcular gall bladder.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra-renal glands.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.
- **Rather stationary appearance** of the previously noted sclerotic & lytic osseous lesions.

OPINION:

**** Reported case of right breast carcinoma showing:**

- **Overall stationary course** since last study.

Dr. Mohamed Mofisen, M.D.



Patient Name	: الأستاذة / اتوار محمد متولي	Request No.	: 139246
Patient No.	: 27509262100601	Request Date	: 28/11/2021
Path No.	: 2926/21	Verification Date	: 06/12/2021
Sex	: Female	Financial Category	: أورام دار السلام - مبانرة
Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:

Rt breast mass

Nature of the specimen:

Tru cut biopsy

Gross Examination:

Four gray white tissue cores up to 1.3cm long, totally submitted.

Microscopic examination:

- Histological type: IDC
- Grade II
- Angioinvasion: no
- Ductal carcinoma in situ: no

Diagnosis:

Rt breast mass, tru cut biopsy:

Invasive duct carcinoma GII

For (ER, PR, Her2, &Ki67)

Prof.Gamal AbdelAty

Dr.Rana Fathy

Verified by :





Patient Name	: الأستاذة / انوار محمد متولي	Request No.	: 139246
Patient No.	: 27509262100601	Request Date	: 28/11/2021
Path No.	: 2926/21	Verification Date	: 23/12/2021
Sex	: Female	Financial Category	: اورام دار السلام - ميلانز
Age	: 45 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Rt breast mass

Nature of the specimen:

Tru cut biopsy

Gross Examination:

Four gray white tissue cores up to 1.3cm long, totally submitted.

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

- Histological type: IDC
- Grade II
- Angioinvasion: no
- Ductal carcinoma in situ: no

Diagnosis:

Rt breast mass, tru cut biopsy:

Invasive duct carcinoma GII

Biomarkers:

ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)

PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)

Her-2: Negative membrane reaction in tumor cells (score 0)

Ki 67: 10 % nuclear reaction

Prof.Gamal AbdelAty

Dr.Rana Fathy

Verified by :





Patient Name	الأستاذة / انوار محمد متولي	Request No.	139246
Patient No.	27509262100601	Request Date	28/11/2021
Path No.	2926/21	Verification Date	23/12/2021
Sex	Female	Financial Category	اورام دار السلام - مياثره
Age	45 Y	Department	Out Patient
Referring Doc.		Registered by	marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Verified by :

Printing Date : 06/01/2025 13:54:47

Page 2 of 2

Printed By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

جمهورية مصر العربية
بطاقة تحقيق الشخصية

النوار
محمد متولى ابوكريم
قرية سقاره
مركز البدرشين - الجيزة

١٩٧٥/٠١/٢١
٢٧٥.٠٩٢٦٢١.٠٠٦.٠١
JY3596721

٢٠٢٢/٠٢
٢٧٥.٩٢٦٢١.٠٠٦.٠١

ربة منزل
أنثى
مسلمة
متزوجة
شاكر محمد مرسى محمد ابوكريم
البطاقة سارية حتى ٢٠٣٠/٠٢/٢٢

١



Name : Anwar Mohammed Metwally Abukrim
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 19-06-2026 10:04
Medical No. : 397426
Ins.Company : تعاقد مستشفيات

Whole-Body Bone Scintigraphy

Clinical Indication: Initial staging of a 51-year-old female with right breast carcinoma, bone pain, and a history of trauma. Currently on hormonal therapy.

Procedure: Anterior and posterior whole-body planar imaging following Tc-99m MDP administration.

Findings:

- **Axial Skeleton:** Mild, heterogeneous radiotracer uptake is noted along the right sternal border, upper thoracic, and lumbar spine. No classic, intensely focal osteoblastic lesions are seen; these areas of heterogeneity remain **equivocal**. Normal physiological uptake throughout the skull, ribs, and pelvis.
- **Appendicular Skeleton & Soft Tissues:** Symmetrical, mild periarticular uptake in the shoulders and knees, consistent with degenerative or post-traumatic changes. Normal renal clearance and urinary excretion without abnormal extraosseous localization.

Impression:

- **Equivocal bone scan:** Mildly heterogeneous, indeterminate tracer distribution along the right sternal border, upper thoracic, and lumbar spine. Differentials include benign degenerative disease, post-traumatic remodeling, or early/atypical metastatic involvement.
- **Recommendation:** Correlative imaging is required to resolve these indeterminate findings.

Dr. Alaa Nagi,
MD Nuclear Medicine

El-Dokki General Hospital
El-Dokki General Hospital
El-Dokki General Hospital



الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي

إسم المريض : أنوار محمد متولى أبو كريم

محول من :

السن : 51 سنة

(2026) (1679 - 2026)

تاريخ الحضور : 2026/07/12

تاريخ الاستلام : 2026/07/25

NATURE OF SPECIMEN :

One paraffin block (Right breast UOQ nodule, tru-cut biopsy).

GROSS :

One paraffin block: sections immunostained for ER, PR, Her-2/neu, ki67, using supersensitive monoclonal antibodies with En-Vision TM+ (HRP) detection system employing peroxidase after antigen retrieval and blocking internal peroxidase and using DAB as chromogen using BenchMark GX automated immunostainer

MICROSCOPY :

Estrogen receptors: Strong nuclear staining in 85 % of tumor cells.
Progesterone receptors: Moderate nuclear staining in 80% of tumor cells.
Her-2/neu: No membranous staining of tumor cells.
Ki67: Positive nuclear staining in about 3% of tumor cells.

DIAGNOSIS :

ER POSITIVE ESTROGEN RECEPTORS, SCORE 8/8.
PR POSITIVE PROGESTERONE RECEPTORS, SCORE 7/8.
HER-2/neu, SCORE 0
Ki67: POSITIVE IN 3% OF TUMOR CELLS.

RECOMENDATION :

Prof. Dr. Fatma A. Mourad, MD.

أ. د. فاطمة أحمد مراد

The General Organization for Teaching
Hospital and Institutes
Ahmed Maher Teaching Hospital



الهيئة العامة للمستشفيات والمعاهد التعليمية

مستشفى أحمد ماهر التعليمي

إسم المريض : انوار محمد متولى ابو كريم

محول من :

السن : 51 سنة

(2026) (1679 - 2026)

تاريخ الاستلام : 2026/07/07

تاريخ الحضور : 2026/06/30

NATURE OF SPECIMEN :

Right breast UOQ nodule, tru-cut biopsy.

GROSS :

Multiple gray white tissue cores, the longest measured 0.8cm long, totally submitted.

MICROSCOPY :

Sections examined revealed mammary tissue cores infiltrated by clusters, nests, indian file pattern and singly dispersed anaplastic cells infiltrating among desmoplastic stroma. Tumor cells show moderate pleomorphism and moderate tendency to form tubules.

DIAGNOSIS :

Right breast UOQ nodule, tru-cut biopsy;
Invasive mixed ductal and lobular carcinoma, grade II.

RECOMENDATION :

Prof. Dr. Fatma A.Mourad, MD.

رقم البطاقة: 27509262100601

٦١٢٤

عبد
المنعم
٦١٢٤

جمهورية مصر العربية
بطاقة تحقيق الشخصية



انوار
محمد متولى ابوكريم
قرية سقاره
مركز البدرشين - الجيزة

١٩٧٥/٠٩/٢١
٢٧٥٠٩٢٦٢١٠٠٦٠١
JY3596721

سونا / ٤ فامو
١٠٢٦٥٣٠٦٤٣

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital



Patient Name: Anwar Mohamed Metwaly

Date: 22/6/2026

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLMENTARY ULTRASOUND

Indication:

- Screening.

Technique:

- CC and MLO views.
- High resolution ultrasound.

Findings:

- Predominant fibro fatty parenchyma. (ACR a).
- Right UOQ area of focal asymmetry measuring about 4.2x2cm with distance form nipple about 9cm.
- No grouped micro-calcifications.
- Complimentary ultrasound showed both breasts are of homogenous parenchymal texture.
- Right UOQ 11 O'clock area of altered parenchyma with small hypoechoic irregular mass showing no internal vascularity measuring about 9x5mm.
- No left cystic or solid masses are detected.
- Normal skin thickness & nipple/areola complexes.
- Nonspecific axillary lymph nodes.

Conclusion:

- Right breast UOQ suspicious lesion as described (BIRADS -4).
- Left breast normal study (BIRADS I)

Recommendations:

- Tru cut biopsy of right breast lesion.

Much Obliged.

Dr. Osama Ismael

Dr. Hagar Mohamed MSC

Name : Anwar Mohamed Metwally Abu Karim
Branch : Maadi
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 22-07-2025 01:44
Medical No. : 157805
Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

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Technique:

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Findings:

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- Degenerative changes are seen at the spine and big joints.

Impression:

- No frank evidence of metastatic bone disease.
- Equivocal tracer uptake pattern at the sternum, for clinical/radiological assessment and follow up.

Much obliged
Dr. Mohammed AtaAllah, MSc
Nuclear Medicine
Dr. Alaa Nagi, MD
Consultant of nuclear medicine

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Medical No. : 397426

Ref.Physician Prof : Goustav Roussey Egypt

تعليق مستشفى : Ins.Company

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Findings:

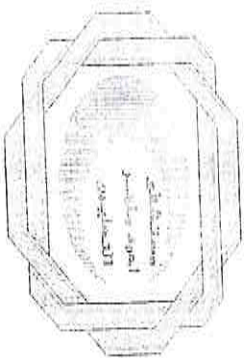
- **Axial Skeleton:** Mild, heterogeneous radiotracer uptake is noted along the right sternal border, upper thoracic, and lumbar spine. No classic, intensely focal osteoblastic lesions are seen; these areas of heterogeneity remain **equivocal**. Normal physiological uptake throughout the skull, ribs, and pelvis.
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Impression:

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- **Recommendation:** Correlative imaging is required to resolve these indeterminate findings.

Dr. Alaa Nagi,

MD Nuclear Medicine



اسم المريض : انوار محمد منولى ابو كرم

محول ص :

السن : 51 سنة

2026) (1679 - 2026)

تاريخ الحضور : 2026/07/12

تاريخ الاستلام : 2026/07/25

NATURE OF SPECIMEN :

core paraffin block (Right breast UOQ nodule, tru-cut biopsy).

GROSS :

core paraffin block sections immunostained for ER, PR, Her-2/neu, ki67, using supersensitive
antibodonal antibodies with En-Vision TM+ (HRP) detection system employing peroxidase
after antigen retrieval and blocking internal peroxidase and using DAB as chromogen using
 BenchMark 3X automated immunostainer

MICROSCOPY :

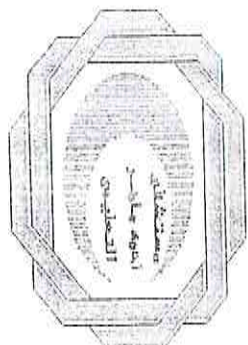
estrogen receptors: Strong nuclear staining in 85 %of tumor cells.
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DIAGNOSIS :

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POSITIVE PROGESTERONE RECEPTORS, SCORE 7/8.
HER-2/neu, SCORE 0
Ki67 POSITIVE IN 3% OF TUMOR CELLS.

RECOMMENDATION :

Dr. Fatma A.Mourad, MD.



اسم المريض : اوار محمد متولى ابو كريم				محول من :	
السن : 51 سنة		تاريخ الاستلام : 2026/07/07		تاريخ الحضور : 2026/06/30	
(	2026	)	(	1679	)

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Right breast UOQ nodule, tru-cut biopsy.

GROSS :

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MICROSCOPY :

Sections examined revealed mammary tissue cores infiltrated by clusters, nests, indian file pattern and singly dispersed anaplastic cells infiltrating among desmoplastic stroma. Tumor cells show moderate pleomorphism and moderate tendency to form tubules.

DIAGNOSIS :

Right breast UOQ nodule, tru-cut biopsy;
Invasive mixed ductal and lobular carcinoma, grade II.

RECOMMENDATION :

Prof. Dr. Fatma A.Mourad, MD.
رقم البطاقة: 07509262100601

71 C 4

Handwritten notes in Arabic script, including "خ" and "١٤١١".



جمهورية مصر العربية
وزارة الداخلية
مصلحة سجلات السكان

انوار
محمد متولى ابو كرم
قرية سقاره
مركز البرشين - البحيره

٢٧٥٠٩٢٦٧١٠٠٦٠١

JY3596721

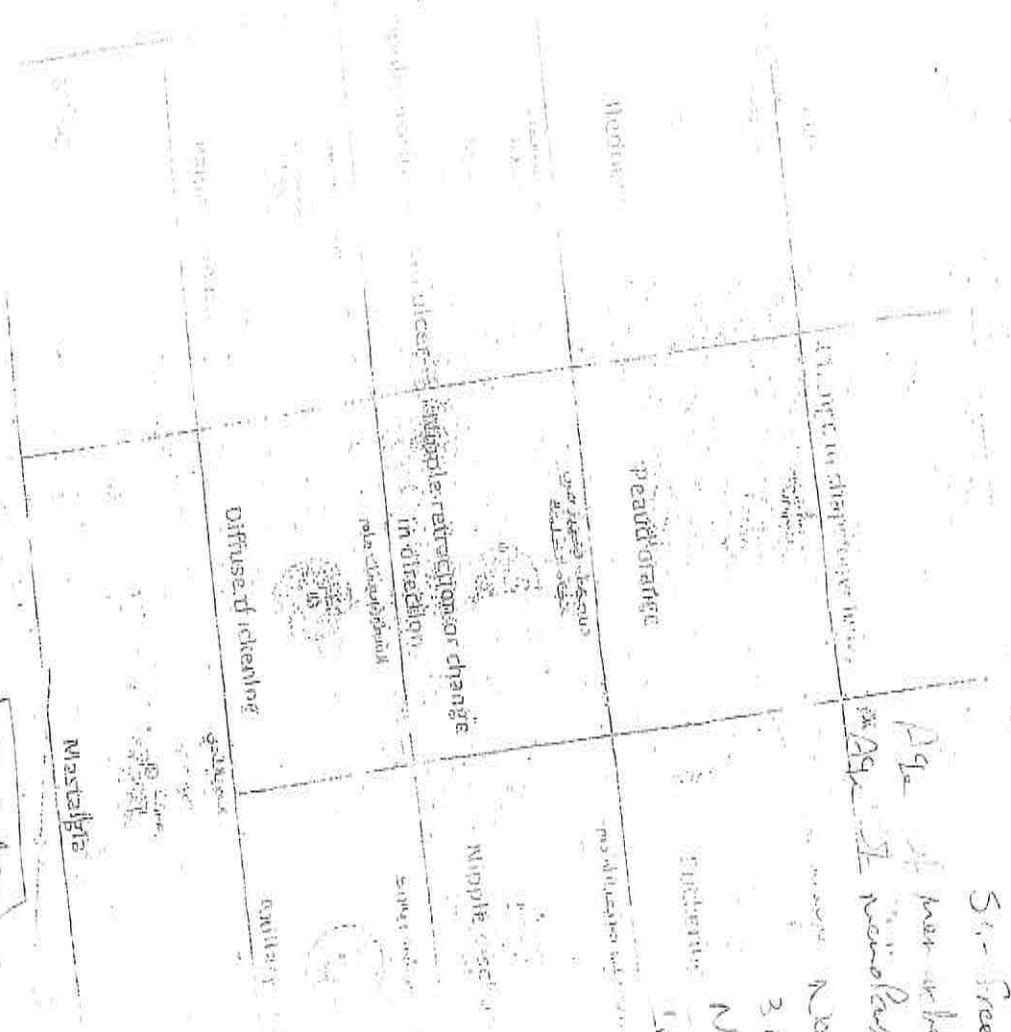
Handwritten notes in Arabic script, including "١٠٣٦٥٣٠٦٤٣" and "١٠٣٦٥٣٠٦٤٣".

Simons - 2/28/2018

My Free
Sx-free

Age 46 - 16 years old
Age 46 - 45 years old
Normal breast feel?
3 children, no breast
Nipple breast feel?
injection for cancer

ve feel 4 Hx
-ve Person Hx



Inspection

Skin dimpling in (R) breast - compatible
with Hx of breast abscess which was
uninfected.
Scar in between 2 breasts
Hx of an abscess.

Palpation

Diffuse bilateral thickening
with no detection of any
lumps.

Central and retroareolar
nipple
trapping study requested
sonography
Mammoscopy
MRI breasts

add up 15.1
new 5/21/2018



Patient Name: *Amwar Mohamed Metwaly*

Date: 22/6/2026

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

- Screening

Technique:

- CC and MLO views.
- High resolution ultrasound.

Findings:

- Predominant fibro fatty parenchyma. (ACR a).
- Right UOQ area of focal asymmetry measuring about 4.2x2cm with distance from nipple about 9cm.
- No grouped micro-calcifications.
- Complementary ultrasound showed both breasts are of homogeneous parenchymal texture.
- Right UOQ 11 O'clock area of altered parenchyma with small hypoechoic irregular mass showing no internal vascularity measuring about 9x5mm.
- No left cystic or solid masses are detected.
- Normal skin thickness & nipple/areola complexes.
- Nonspecific axillary lymph nodes.

Conclusion:

- Right breast UOQ suspicious lesion as described (BIRADS -4).
- Left breast normal study (BIRADS I)

Recommendations:

- True cut biopsy of right breast lesion.

Mehd Odeged

Dr. Osama Youns

Dr. Hagar Mohamed MSE

Name : Anwar Mohamed Metwally Abu Karim
Branch : Maadi
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 22-07-2025 01:44
Medical No. : 157805
التأمين الصحي : Ins.Company

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 50-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Diffuse enhanced tracer uptake is noted at the sternal body.
- No other abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- No frank evidence of metastatic bone disease.
- Equivocal tracer uptake pattern at the sternum, for clinical/radiological assessment and follow up.

Much obliged

*Dr. Mohammed Atallah, MSc
Nuclear Medicine*

*Dr. Alaa Nagi, MD
Consultant of nuclear medicine*